

1. Administrative & Study Identification

Full Study Title: Study to Evaluate Tulisokibart for Hidradenitis Suppurativa (MK-7240-012) **Short Title/Acronym:** MK-7240-012
Protocol Number: MK-7240-012 **NCT Number:** NCT06956235 **Sponsor Name:** Merck Sharp & Dohme LLC **Investigational Product:** Tulisokibart (Anti-TL1A monoclonal antibody) **Phase of Development:** Phase 2b **Medical Monitor:** Merck Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate whether at least 1 dose level of tulisokibart is superior to placebo with respect to the proportion of participants achieving a 50% reduction in Hidradenitis Suppurativa Clinical Response (HiSCR50) at Week 16 (end of the double-blind treatment period). **Secondary Objectives:** To assess the long-term safety, tolerability, and sustained efficacy of tulisokibart over a 100-week Long-term Extension (LTE) period.

2.2 Background & Rationale To address the medical necessity of treating moderate to severe hidradenitis suppurativa (HS), a chronic inflammatory skin condition. There remains a significant unmet need for treatments that successfully manage HS, as standard antibiotic and biologic therapies can be inadequate. Tulisokibart targets the TNF-like cytokine 1A (TL1A) pathway, which is heavily associated with both tissue inflammation and fibrosis, directly addressing the underlying pathogenesis and fibrotic complications of HS.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** Randomized (Parallel Group)

3.2 Planned Enrollment & Duration Target \$N\$: 147 participants
Number of Sites: Global (Including United States, Australia, Canada, China, France, Germany, Italy, Japan, Netherlands, Spain, United Kingdom, etc.) **Estimated Study Start:** 09-Jun-2025
Estimated Primary Completion: 16-Nov-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age 18 to 75 years old. 2. Has signs and symptoms of hidradenitis suppurativa (HS) for ≥ 6 months prior to Screening and a clinical diagnosis of HS at Screening. 3. Has moderate or severe HS defined as a total combined number of ≥ 5 abscesses and/or inflammatory nodules, with HS lesions present in ≥ 2 distinct anatomical areas and ≥ 1 anatomic area of HS involvement characterized as Hurley Stage II or III.

4.2 Exclusion Criteria 1. Has a history of inadequate response to a course of systemic antibiotics for treatment of HS or intolerance to or has a contraindication to systemic antibiotics for treatment of HS. 2. Has >20 draining tunnel count at Screening and Randomization. 3. Has undergone laser therapy or surgical procedures for their lesions within the past 6 weeks or is planning to have laser therapy or surgical procedures for lesions during participation in the study.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Participants are randomized to receive one of 3 dose regimens (High Dose, Medium Dose, or Low Dose) of Tulasikibart or Placebo. **Route of Administration:** Subcutaneous (SC) injection via autoinjector. **Duration of Treatment:** 16-week Double-blind Period, followed by a 100-week Long-term Extension (LTE) composed of a 40-week Main Extension and a 60-week Optional Extension.

5.2 Concomitant & Prohibited Therapy Permitted: Stable background care and non-prohibited medications allowed for co-morbidities. **Prohibited:** Laser therapy or surgical procedures for HS lesions during the trial, and systemic treatments conflicting with the mechanism of action of the investigational product.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Baseline Lesion Count
Baseline	Day 0	Randomization, First Dose Administration via SC Autoinjector
Week 4	Week 4	Onsite Visit, Clinical Examination, Safety Monitoring
Week 8	Week 8	Onsite Visit, Clinical Examination, Safety Monitoring
Week 12	Week 12	Onsite Visit, Clinical Examination, Safety Monitoring
Interim	Week 16	Primary Efficacy Assessment (HiSCR50 evaluation), End of Double-Blind Phase
LTE Visits	Weeks 20 to 115	Scheduled extension interval visits, Remote Monitoring Q4W, Safety Checklist Review Q12W
End of Study	Week 116	Final Vital Signs, Exit Interview, Long-Term Extension (LTE) Completion

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Proportion of participants achieving a 50% reduction in Hidradenitis Suppurativa Clinical Response (HiSCR50) at Week 16. **Measurement Method:** Clinical examination measuring $\geq 50\%$ reduction in the total abscess and inflammatory nodule count with no increase in abscess count and no increase in draining fistula count relative to baseline.

7.2 Secondary & Exploratory Endpoints * Maintenance of HiSCR50 response during the LTE period up to 116 weeks. * Safety and tolerability endpoints (incidence of adverse events).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Routine collection via HCP assessment, physical examinations, and patient reporting at scheduled visits. **Serious Adverse Events (SAEs):** Reported by the investigator to the sponsor within 24 hours of site awareness. **Data Monitoring Committee (DMC):** Internal/External committee governed by Merck Sharp & Dohme LLC.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) Set for efficacy hypotheses; Safety Set for all participants receiving ≥ 1 dose of the study drug. **Sample Size Justification:** $N = 147$ statistically powered ($\alpha = 0.05$) to establish superiority of at least one Tulisokibart dose level over placebo regarding HiSCR50 at Week 16. **Handling of Missing Data:** Non-Responder Imputation (NRI) for primary binary categorical efficacy variables.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Scheduled onsite and remote clinical monitoring to ensure protocol adherence and patient safety. **Audit Trail:** Robust 21 CFR Part 11 compliant eCRF locking and electronic data clarification forms.

11. Study Identifiers & Governance

Primary ID: MK-7240-012 **Registry Number:** NCT06956235 **Sponsor/Lead Organization:** Merck Sharp & Dohme LLC **Study Title:** Study to Evaluate Tulisokibart for Hidradenitis Suppurativa (MK-7240-012) **Official Regulatory Title:** A phase 2b randomized, double-blind,

placebo-controlled study of the safety and efficacy of
tulisokibart in participants with moderate to severe hidradenitis
suppurativa

12. Clinical Framework (Hidradenitis Suppurativa Specific)

Primary Condition: Moderate to Severe Hidradenitis Suppurativa (HS) **Diagnosis Threshold:** ≥ 5 abscesses and/or inflammatory nodules, Hurley Stage II or III **Medical Methodology:** Anti-TL1A monoclonal antibody therapy (Tulisokibart) **Optimization Target:** HiSCR50 Response at 16 weeks

13. Study Procedures & Methodology

Management Pathway: Dermatological assessment, lesion mapping, and abscess/nodule/tunnel counting **Education Protocol (HCP):** Protocol training on HiSCR measurement criteria and autoinjector utilization **Education Protocol (Patient):** Self-administration training for the SC autoinjector **Quality Audit Mechanism:** Centralized evaluation of recorded clinical responses and AE tracking

14. Observation & Follow-Up Schedule

Total Duration: 116 weeks total (16 weeks double-blind + 100 weeks long-term extension) **Onsite Visit Cadence:** Weeks 0, 4, 8, 12, 16, and scheduled extension interval visits **Remote Monitoring Frequency:** Q4W (every 4 weeks) where clinical visits do not occur **Checklist Review Interval:** Every 12 weeks (Q12W) to review long-term safety

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				